

Supplementary appendix

Supplement to: Allen JA, Lin J, Basta I, et al. Safety, tolerability, and efficacy of subcutaneous efgartigimod in patients with chronic inflammatory demyelinating polyradiculoneuropathy (ADHERE): a multicentre, randomised-withdrawal, double-blind, placebo-controlled, phase 2 trial.

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Full inclusion and exclusion criteria

Inclusion criteria

1. Ability to understand the requirements of the trial, provide written informed consent (including consent for the use and disclosure of research-related health information), willingness and ability to comply with the trial protocol procedures (including required trial visits).
2. Male or female patient aged 18 years or older at the time of signing the informed consent.
3. Diagnosed with probable or definite chronic inflammatory demyelinating polyradiculoneuropathy (CIDP) according to criteria of the 2010 European Federation of Neurological Societies/Peripheral Nerve Society (EFNS/PNS), progressing or relapsing forms.
4. CIDP Disease Activity Status score ≥ 2 at screening.
5. An Inflammatory Neuropathy Cause and Treatment (INCAT) score ≥ 2 , with a score of 2 exclusively from leg disability, at the first run-in visit (RI-V1; for patients entering run-in) or stage A baseline (for off-treatment patients with documented evidence for worsening on the total adjusted [a]INCAT disability score within 3 months prior to screening).
6. Fulfilling any of the following treatment conditions:
 - Currently treated with pulsed corticosteroids, oral corticosteroids equivalent to prednisolone/prednisone ≤ 10 mg/day, and/or intravenous immunoglobulin (IVIg) or subcutaneous immunoglobulin (SCIg), if the patient is willing to discontinue this treatment at the RI-V1; OR
 - Off treatment: Without previous treatment; OR treatment with corticosteroids and/or IVIg or SCIg discontinued ≥ 6 months prior to screening.

Note: Patients not treated with monthly or daily corticosteroids, IVIg, or SCIg for ≥ 6 months prior to screening were considered as equal to off-treatment patients.
7. Women of childbearing potential who had a negative pregnancy test at screening and a negative urine pregnancy test up to stage A baseline.
8. Women of childbearing potential must use a highly effective method of contraception (failure rate of less than 1% per year) from screening to 90 days after the last administration of investigational medicinal product (IMP).
9. Non-sterilised male patients who were sexually active with a female partner of childbearing potential must use a condom, and his partner must use a highly effective method of contraception (failure rate of less than 1% per year) from screening to 90 days after the last administration of IMP. Male patients practicing true sexual abstinence (when this is in line with the preferred and usual lifestyle of the participant) could be included. Sterilised male patients who have had vasectomy with documented aspermia post procedure could be included. In addition, male patients were not allowed to donate sperm from screening to 90 days after the last administration of IMP.

Exclusion criteria

1. Pure sensory atypical CIDP (EFNS/PNS definition).
2. Polyneuropathy of other causes, including the following:

- Multifocal motor neuropathy;
 - Monoclonal gammopathy of uncertain significance with anti–myelin-associated glycoprotein immunoglobulin M antibodies;
 - Hereditary demyelinating neuropathy;
 - Polyneuropathy, organomegaly, endocrinopathy, monoclonal protein, and skin change syndromes;
 - Lumbosacral radiculoplexus neuropathy;
 - Polyneuropathy most likely due to diabetes mellitus;
 - Polyneuropathy most likely due to systemic illnesses;
 - Drug- or toxin-induced polyneuropathy.
3. Any other disease that could better explain the patient’s signs and symptoms.
 4. Any history of myelopathy or evidence of central demyelination.
 5. Current or past history (within 12 months of screening) of alcohol, drug, or medication abuse.
 6. Severe psychiatric disorder (such as severe depression, psychosis, bipolar disorder), history of suicide attempt, or current suicidal ideation that in the opinion of the investigator could create undue risk to the patient or could affect adherence with the trial protocol.

Note: At screening, suicidality was assessed using the Columbia-Suicide Severity Rating Scale (C-SSRS); patients with a high suicide risk were excluded from the trial (ie, patients will be excluded with a positive answer to questions #4 and/or #5 of the suicidal ideation subscale [over the past 3 months]; and/or any positive answer to the suicidal behavior subscale [over the past year]). Any positive answer to the above questions under “Lifetime/Time he/she felt most suicidal” should be carefully evaluated for any current risk of suicide by the investigator prior to trial entry.
 7. Patients with clinically significant active or chronic uncontrolled bacterial, viral, or fungal infection at screening, including patients who tested positive for an active viral infection at screening with:
 - Active hepatitis B Virus: serologic panel test results indicative of an active (acute or chronic) infection;
 - Active hepatitis C Virus (HCV): serology positive for HCV-Ab;
 - Human immunodeficiency virus–positive serology associated with an acquired immune deficiency syndrome–defining condition or with a cluster of differentiation 4 count ≤ 200 cells/mm³.
 8. Total immunoglobulin G level <6 g/L at screening.
 9. Treatment with the following:
 - Within 3 months (or 5 half-lives of the drug, whichever is longer) before screening: plasma exchange or immunoadsorption, any concomitant Fc-containing therapeutic agents or other biological, or any other investigational product;
 - Within 6 months before screening: rituximab, alemtuzumab, any other monoclonal antibody, cyclophosphamide, interferon, tumor necrosis factor-alpha inhibitors, fingolimod, methotrexate, azathioprine, mycophenolate, any other immunomodulating or immunosuppressive medications, and oral daily corticosteroids >10 mg/day.

Note: Patients using IVIg, SCIg, pulsed corticosteroids, and oral daily corticosteroids ≤ 10 mg/day could be included.

10. Pregnant and lactating women and those intending to become pregnant during the trial or within 90 days after last IMP administration.
11. Patients with any other known autoimmune disease that, in the opinion of the investigator, would interfere with an accurate assessment of clinical symptoms of CIDP.
12. Patients who received a live-attenuated vaccine fewer than 28 days before screening. Receiving an inactivated, sub-unit, polysaccharide, or conjugate vaccine any time before screening was not exclusionary.
13. Patients who had a history of malignancy unless deemed cured by adequate treatment with no evidence of recurrence for ≥ 3 years before the first IMP administration. Patients with the following cancer could be included anytime:
 - Adequately treated basal cell or squamous cell skin cancer,
 - Carcinoma in situ of the cervix,
 - Carcinoma in situ of the breast, or
 - Incidental histological finding of prostate cancer (tumor, nodes, and metastases classification stage T1a or T1b).
14. Patients who previously participated in a trial with efgartigimod and had received ≥ 1 administration of IMP.
15. Patients with known medical history of hypersensitivity to any of the ingredients of IMP.
16. Patients with clinical evidence of other significant serious disease or patients who underwent a recent or had a planned major surgery, or any other reason which could confound the results of the trial or put the patient at undue risk.

aINCAT, I-RODS, and grip strength

INCAT is a 10-point scale that evaluates the functionality of the arms and legs. Scores range from 0–10 (with combined scores from 0–5 for each of arm disability and leg disability), with 10 indicating the greatest degree of disability. For the aINCAT scale, changes in arm function from 0–1 or 1–0 are not considered deterioration or improvement, respectively, because these changes are not considered clinically significant.

Inflammatory Rasch-built Overall Disability Scale (I-RODS) is a 24-item, participant-reported scale with each item representing a common daily activity or social participation that ranges from very easy to very difficult, with higher scores indicating less disability. Raw sum scores of the 24-item I-RODS score (ranging from 0–48) are converted to a centile metric score ranging from 0–100, with lower scores indicating the greatest degree of disability.

Mean grip strength was assessed with a handheld Martin vigorimeter. Participants perform 3 assessments for each hand in an arbitrary order (with ~30-second rest between each assessment) at approximately the same time during the day. The mean grip strength for each hand is used to determine impairment. Lower mean grip strength indicates greater impairment.

Definition of Confirmed ECI endpoint in stage A

- For off-treatment participants or for non-off-treatment participants who deteriorated in aINCAT score during the run-in period (increase of ≥ 1 points versus the first visit of the run-in), ECI in stage A was defined as a confirmed improvement in the aINCAT score (decrease of ≥ 1 points vs stage A baseline).
- For non-off-treatment participants who had no change in aINCAT score during run-in and deteriorated on I-RODS and/or mean grip strength during run-in, ECI could also be based on a confirmed improvement on I-RODS (increase of ≥ 4 points vs stage A baseline) and/or mean grip strength (increase of ≥ 8 kPa vs stage A baseline), depending on whether clinical deterioration during run-in was based on I-RODS, mean grip strength, or both.
- Confirmed ECI was defined as fulfilling the criteria for ECI at 2 consecutive visits (on the same efficacy parameter and with no missed visits in between) and the participant having received ≥ 4 administrations of efgartigimod PH20 SC.

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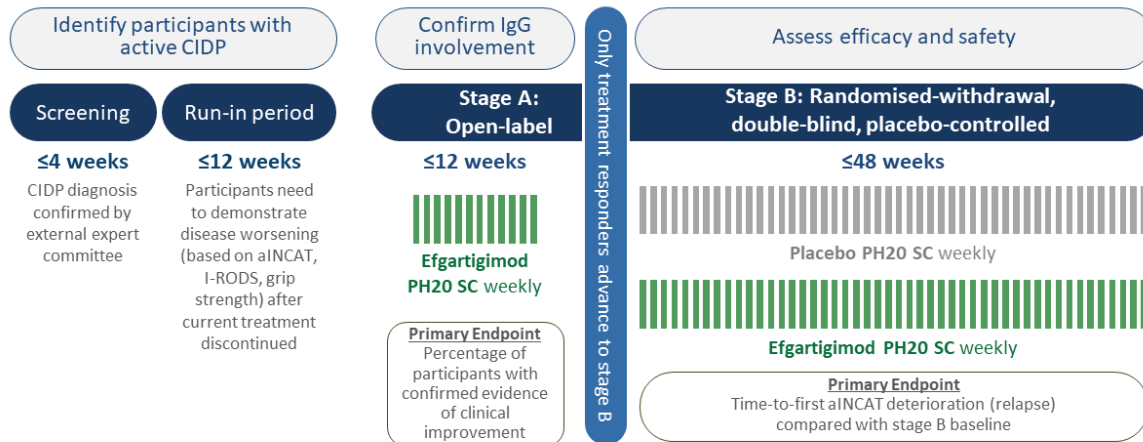
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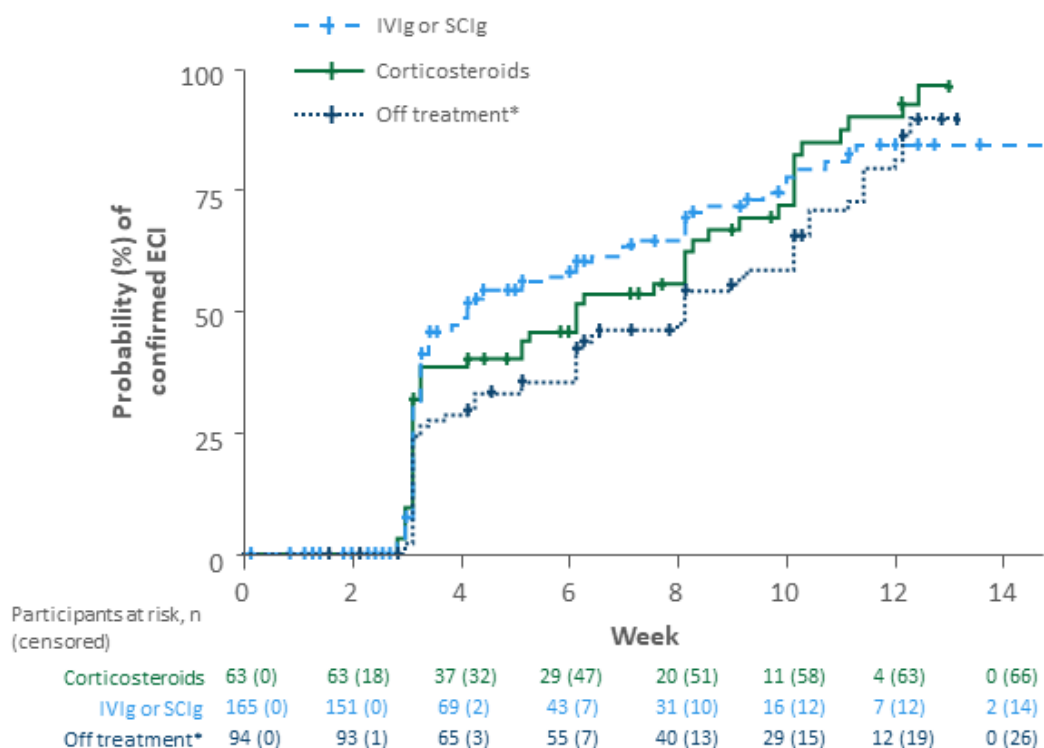
Christina Ulane Louis Weimer Thomas Adam Purvis Nathan Carberry		
Shan Chen Ram Gowda David Atherton Rasha Waheed Pengfei Zhang	Rutgers University – Robert Wood Johnson Medical School	United States
Jonathan Katz	California Pacific Medical Center	United States
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Pantelis Pavlakis Dale Lange Shara Holzberg Dora Leung Nicole Frand Alexa Bulavinetz	Hospital for Special Surgery	United States
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Karissa Gable Shruti Raja Jordan Mayberry Xiaoyan Li Vern Juel Natalia Gonzalez Lisa Hobson-Webb	Duke University School of Medicine	United States
Michael Rivner Benjamin Barnes	Augusta University	United States
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Sadiq Altamimi	The Neurology Group	United States
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Supplementary figure 1: Trial design

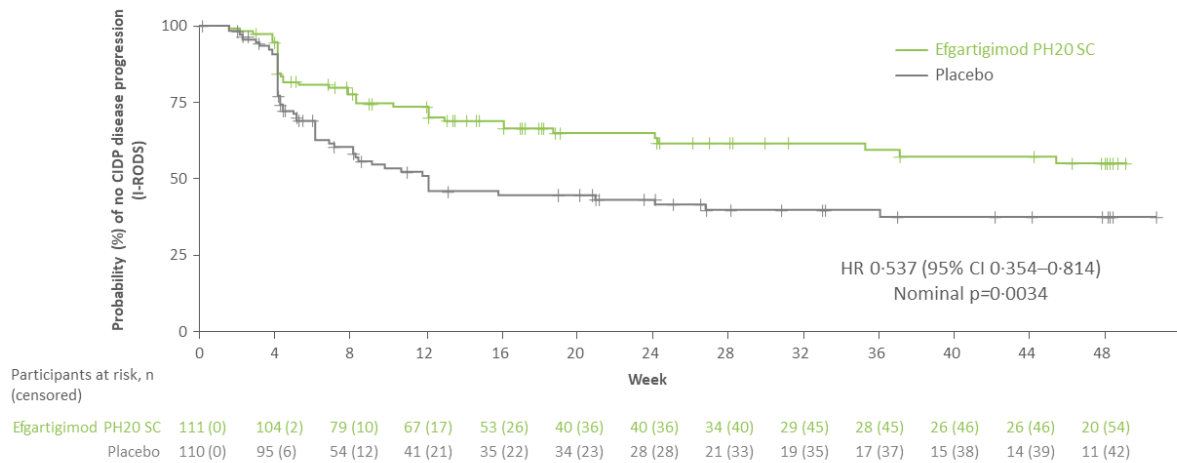
aINCAT=adjusted Inflammatory Neuropathy Cause and Treatment. CIDP=chronic inflammatory demyelinating polyradiculoneuropathy. IgG=immunoglobulin G. I-RODS=Inflammatory Rasch-built Overall Disability Scale. PH20=recombinant human hyaluronidase PH20. SC=subcutaneous.



Supplementary figure 2: Kaplan-Meier analysis of time to initial confirmed ECI by prior CIDP treatment (secondary analysis; stage A; safety population)

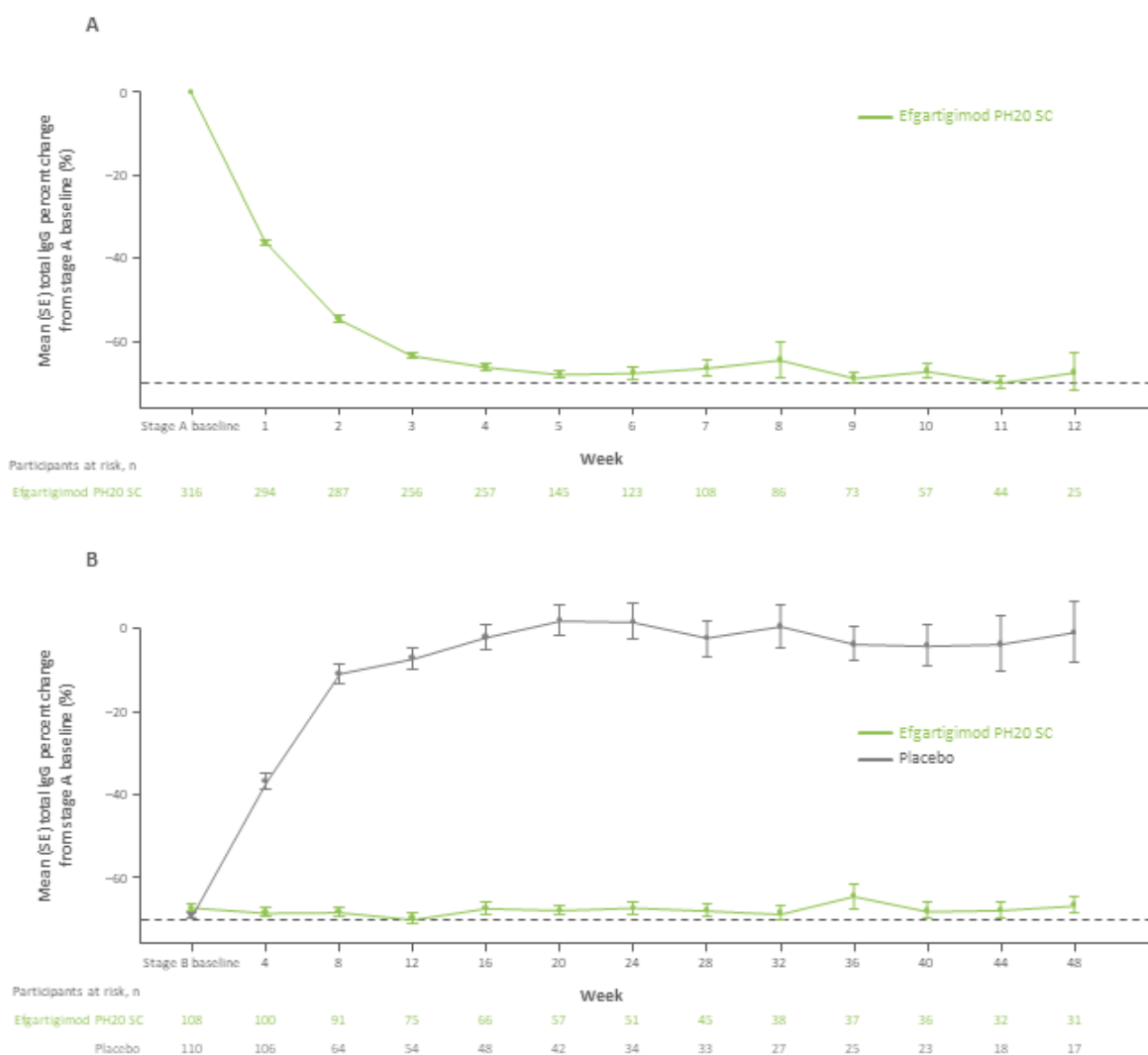
The stage A safety population included all participants who received ≥ 1 doses or part of a dose of the investigational medicinal product in stage A. CIDP=chronic inflammatory demyelinating polyradiculoneuropathy. ECI=evidence of clinical improvement. IVIg=intravenous immunoglobulin. SCIg=subcutaneous immunoglobulin.

*Off treatment was defined as participants who had never received CIDP treatment (treatment naïve) or who had not received CIDP treatment (corticosteroids, IVIg, or SCIg) within 6 months of study entry.



Supplementary figure 3: Kaplan-Meier analysis of time to CIDP disease progression (secondary analysis; stage B; mITT population)

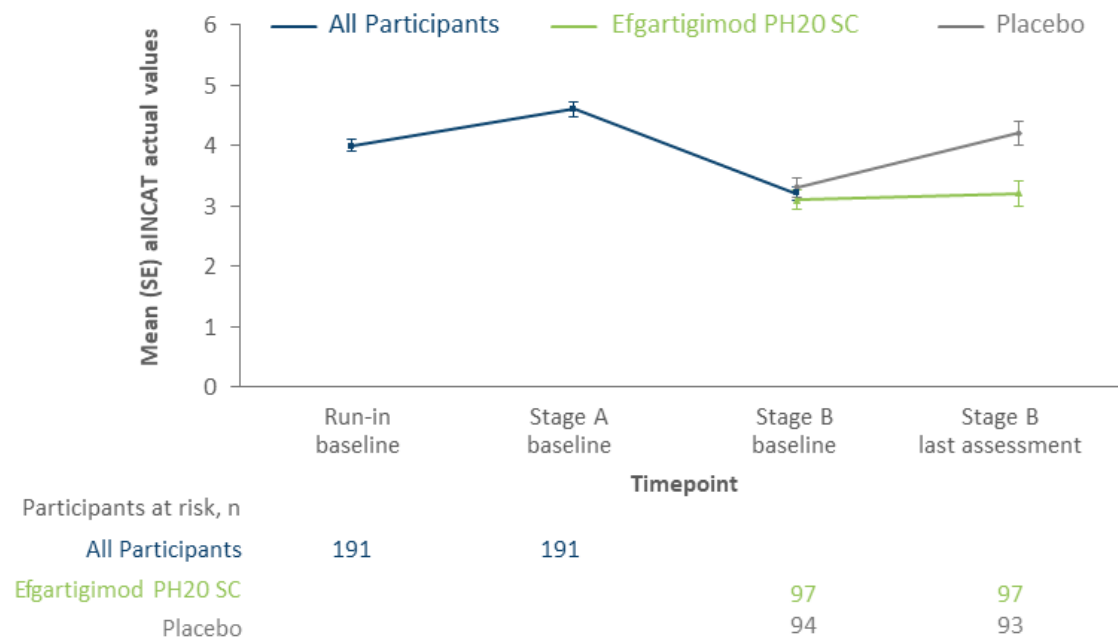
The mITT population included all randomised participants who received ≥ 1 doses or part of a dose of the investigational medicinal product in stage B. The HR was obtained from a Cox proportional hazard model with treatment as a fixed effect, and the model was stratified by prior CIDP therapy and aINCAT score during stage A. aINCAT=adjusted Inflammatory Neuropathy Cause and Treatment. CIDP=chronic inflammatory demyelinating polyradiculoneuropathy. I-RODS=Inflammatory Rasch-built Overall Disability Scale. HR=hazard ratio. mITT=modified intent-to-treat. PH20=recombinant human hyaluronidase PH20. SC=subcutaneous.



Supplementary figure 4: Percent change in total IgG over time from stage A baseline in (A) stage A and (B) stage B (secondary analysis; pharmacodynamic analysis set)

The pharmacodynamic population included all participants in the safety population for whom at least one serum pharmacodynamic concentration was available. Stages A and B baseline were defined as the last available value before the first administration of the investigational medicinal product in stages A and B, respectively.

IgG=immunoglobulin G. PH20=recombinant human hyaluronidase PH20. SC=subcutaneous.



Supplementary figure 5: Longitudinal mean aINCAT scores of stage A responders from run-in baseline to last assessment in stage B (post-hoc analysis; run-in set and mITT populations)

The run-in set and mITT populations included participants with a run-in baseline assessment who were stage A responders and randomised in stage B and received ≥ 1 doses or part of a dose of the investigational medicinal product in stage B. Stages A and B baseline were defined as the last available value before the first administration of the investigational medicinal product in stages A and B, respectively, and stage B last assessment as the last nonmissing, postbaseline value in stage B. aINCAT=adjusted Inflammatory Neuropathy Cause and Treatment. mITT=modified intent-to-treat. PH20=recombinant human hyaluronidase PH20. SC=subcutaneous.

Supplementary table 1: Confirmed ECI (primary and secondary analyses; stage A; safety population)

	Efgartigimod PH20 SC			
Overall	Primary analysis (n=322)	Prior corticosteroids (n=63)	Prior IVIg or SCIg (n=165)	Off treatment* (n=94)
Confirmed ECI	214 (67%) (61·0–71·6)	49 (78%) (65·5–87·3)	97 (59%) (50·9–66·4)	68 (72%) (62·2–81·1)
Median time to initial confirmed ECI, days	43·0 (31·0–51·0)	43·0 (23·0–58·0)	29·0 (24·0–43·0)	57·0 (43·0–71·0)

Data are n (%) or median (95% CI). The stage A safety population included all participants who received ≥ 1 doses or part of a dose of the investigational medicinal product in stage A. The exact 2-sided Clopper-Pearson 95% CI for the ECI responder rate is shown for each prior CIDP therapy subgroup. CIDP=chronic inflammatory demyelinating polyradiculoneuropathy. ECI=evidence of clinical improvement. IVIg=intravenous immunoglobulin. PH20=recombinant human hyaluronidase PH20. SC=subcutaneous. SCIg=subcutaneous immunoglobulin. *Off treatment was defined as participants who had never received CIDP treatment (treatment naïve) or who had not received CIDP treatment (corticosteroids, IVIg, or SCIg) within 6 months of study entry.

Supplementary table 2: Change from stage A baseline to stage A last assessment in clinical efficacy endpoints (secondary and post-hoc analyses; stage A; safety population)

	Efgartigimod PH20 SC (N=322)		
	Overall	ECl responders	ECl nonresponders
aINCAT score	−0.9 (1.71)	−1.4 (1.54)	0.2 (1.49)
I-RODS score	7.7 (15.48)	12.0 (14.62)	−0.7 (13.61)
Grip strength (dominant hand), kPa	12.3 (18.68)	17.9 (16.98)	0.8 (16.71)
Grip strength (nondominant hand), kPa	11.2 (21.12)	17.2 (19.34)	−1.3 (19.15)
MRC Sum score	3.8 (7.24)	5.9 (6.06)	−0.5 (7.53)
TUG test score, s	−4.3 (14.01)	−6.1 (11.47)	−0.4 (17.86)

Data are mean (SD). The stage A safety population included all participants who received ≥ 1 doses or part of a dose of the investigational medicinal product in stage A. Stage A baseline was defined as the last available value before the first administration of the investigational medicinal product in stage A, and stage A last assessment was defined as the last nonmissing, postbaseline value in stage A. aINCAT=adjusted Inflammatory Neuropathy Cause and Treatment. ECl=evidence of clinical improvement. I-RODS=Inflammatory Rasch-built Overall Disability Scale. MRC=Medical Research Council. PH20=recombinant human hyaluronidase PH20. SC=subcutaneous. TUG=Timed Up and Go.

Supplementary table 3: Clinical deterioration rates (primary analysis; stage B; mITT population)

	Efgartigimod PH20 SC	Placebo
Overall	31/111 (28%)	59/110 (54%)
Prior corticosteroids	5/24 (21%)	15/23 (65%)
Prior IVIg or SCIg	14/48 (29%)	28/48 (58%)
Off treatment*	12/39 (31%)	16/39 (41%)
Change of aINCAT score during stage A (randomisation)		
aINCAT score decrease of ≥ 1 points during stage A	24/75 (32%)	41/75 (55%)
No decrease in aINCAT score during stage A	7/36 (19%)	18/35 (51%)

Data are n/N (%). The mITT population included all randomised participants who received ≥ 1 doses or part of a dose of the investigational medicinal product in stage B. aINCAT=adjusted Inflammatory Neuropathy Cause and Treatment. CIDP=chronic inflammatory demyelinating polyradiculoneuropathy. IVIg=intravenous immunoglobulin. mITT=modified intent-to-treat. n=number of participants with clinical deterioration per subgroup. N=number of participants per subgroup in the mITT. PH20=recombinant human hyaluronidase PH20. SC=subcutaneous. SCIg=subcutaneous immunoglobulin. *Off treatment was defined as participants who had never received CIDP treatment (treatment naïve) or who had not received CIDP treatment (corticosteroids, IVIg, or SCIg) within 6 months of study entry.

Supplementary table 4: Detailed summary of TEAEs (secondary analysis; safety population)

	Stage A			Stage B					
	Efgartigimod PH20 SC (N=322) PYFU=46·9			Efgartigimod PH20 SC (N=111) PYFU=56·7			Placebo (N=110) PYFU=42·1		
	n (%)	m	ER [‡]	n (%)	m	ER [‡]	n (%)	m	ER [‡]
Any TEAE	204 (63%)	630	13·4	71 (64%)	197	3·5	62 (56%)	215	5·1
Any grade ≥3 TEAE	25 (8%)	29	0·6	7 (6%)	8	0·1	7 (6%)	9	0·2
Any serious TEAE	21 (7%)	24	0·5	6 (5%)	8	0·1	6 (6%)	8	0·2
Any treatment-related TEAE*	101 (31%)	260	5·5	27 (24%)	61	1·0	22 (20%)	65	1·5
Injection site reaction	62 (19%)	124	2·6	16 (14%)	22	0·4	7 (6%)	9	0·2
Headache	16 (5%)	28	0·6	4 (4%)	6	0·1	2 (2%)	2	0·05
Infections and infestations	44 (14%)	57	1·2	35 (32%)	43	0·8	37 (34%)	53	1·3
COVID-19	7 (2%)	8	0·2	19 (17%)	20	0·4	14 (13%)	14	0·3
Discontinued due to TEAEs	22 (7%)	22	0·5	3 (3%)	3	0·1	1 (1%)	1	0·02
Death [†]	2 (1%)	2	0·04	0	0	0	1 (1%)	1	0·02

Data are n (%). The stages A and B safety populations included all participants who received ≥1 doses or part of a dose of the investigational medicinal product in stages A and B, respectively. CIDP=chronic inflammatory demyelinating polyradiculoneuropathy. COVID-19=coronavirus disease 2019. ER=event rate. m=number of events. PH20=recombinant human hyaluronidase PH20. PYFU=participant years of follow-up. SC=subcutaneous. TEAE=treatment-emergent adverse event. *Treatment related was defined as at least possibly related to the investigational medical product, according to the investigator, or a missing relatedness. [†]Fatal outcomes included 2 in stage A considered not related (cardiac arrest following COVID-19) and unlikely related to treatment (CIDP worsening) and 1 in stage B considered possibly related (pneumonia), but after unblinding was a placebo participant. [‡]ER was calculated as the number of events divided by the PYFU.

Supplementary table 5: Most common TEAEs in stage A (reported in ≥5 participants) (secondary analysis; safety population)

	Efgartigimod PH20 SC (N=322)
Any TEAE*	204 (63%)
Injection site erythema	33 (10%)
CIDP	17 (5%)
Headache	16 (5%)
Injection site pain	15 (5%)
Injection site rash	11 (3%)
Rash	11 (3%)
Upper respiratory tract infection	11 (3%)
Arthralgia	9 (3%)
Diarrhoea	8 (3%)
Pain in extremity	8 (3%)
Back pain	7 (2%)
COVID-19	7 (2%)
Alanine aminotransferase increased	6 (2%)
Myalgia	6 (2%)
Fatigue	5 (2%)
Injection site reaction	5 (2%)
Peripheral swelling	5 (2%)
Nasopharyngitis	5 (2%)
Urinary tract infection	5 (2%)
Fall	5 (2%)
Activated partial thromboplastin time prolonged	5 (2%)
Aspartate aminotransferase increased	5 (2%)
Blood triglycerides increased	5 (2%)

Data are n (%). The stage A safety population included all participants who received ≥1 doses or part of a dose of the investigational medicinal product in stage A. CIDP=chronic inflammatory demyelinating polyradiculoneuropathy. COVID-19=coronavirus disease 2019. PH20=recombinant human hyaluronidase PH20. SC=subcutaneous. TEAE=treatment-emergent adverse event. *Medical Dictionary for Regulatory Activities version 25.1 Preferred Terms.

Supplementary table 6: Most common TEAEs in stage B (reported in ≥3 participants in either arm) (secondary analysis; safety population)

	Efgartigimod PH20 SC (N=111)	Placebo (N=110)
Any TEAE*	71 (64%)	62 (56%)
COVID-19	19 (17%)	14 (13%)
Injection site bruising	6 (5%)	1 (1%)
Injection site erythema	6 (5%)	0
Nasopharyngitis	5 (5%)	3 (3%)
Arthralgia	4 (4%)	5 (5%)
Headache	4 (4%)	2 (2%)
Fatigue	3 (3%)	1 (1%)
Influenza-like illness	3 (3%)	0
Pyrexia	3 (3%)	4 (4%)
Pain in extremity	2 (2%)	4 (4%)
Upper respiratory tract infection	2 (2%)	11 (10%)
Pneumonia	1 (1%)	4 (4%)
Constipation	1 (1%)	3 (3%)
Muscle spasms	0	3 (3%)
Paraesthesia	0	3 (3%)

Data are n (%). The stage B safety population included all participants who received ≥1 doses or part of a dose of the investigational medicinal product in stage B. COVID-19=coronavirus disease 2019. PH20=recombinant human hyaluronidase PH20. SC=subcutaneous. TEAE=treatment-emergent adverse event. *Medical Dictionary for Regulatory Activities version 25.1 Preferred Terms.